

# WEEK 8: INTRO TO PT & MED TERM

## PHYSICAL THERAPY SETTING I (OUTPATIENT, HOSPITALS, ACUTE, INPATIENT)

### OUTPATIENT

- Most well-known setting
  - Setting: private clinics, PT offices, sports facilities
  - Patients: patients with musculoskeletal issues, pelvic health needs, lymphedema, and post-rehabilitation
  - Typical cases: orthopedic injuries (ACL repair, rotator cuff), chronic pain (arthritis), sports injuries, neurological conditions (mild stroke rehab)
  - Focus: manual therapy, exercise program, education, return to function
- Available for all ages
  - Medically stable individuals who can manage their daily activities with or without assistance
- Involves having patients come into a clinic 1-3 times per week to meet with their PT and returning home after each visit
- Flexible and convenient in terms of scheduling therapy sessions around personal commitments
- Self-motivation is required to attain success

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### HOSPITALS

- Most common setting for acute care therapy
- Patients who have undergone surgery, experienced a sudden illness, or have a serious injury may receive therapy services directly in the hospital

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### ACUTE

#### ACUTE CARE

- Hospital setting
  - Early intervention for patients who have been admitted
  - Focuses on immediate needs and stabilization
  - Fast-paced, interdisciplinary team approach
- PTs can be found throughout the hospital on cardiopulmonary, oncology, neurology, orthopedic, and intensive care units
  - Patients: medically unstable or post-surgery patients

- Typical cases: post surgery (hip replacement, cardiac surgery), ICU patients (mobility, pulmonary hygiene), neurological events (recent stroke)

#### - Goals:

- Assess their functional mobility levels
- Provide appropriate exercises
- Instruct gait and mobility training
- Facilitate safe discharge planning to home or the next level of care

### ACUTE REHABILITATION

- Skilled nurse facility (SNF) / long-term acute care (LTAC)
  - Setting: nursing homes, specialized facilities
  - Patients: elderly or chronically ill patients needing long-term rehab
  - Focus: gradual recovery, maintenance therapy, sometimes palliative care
- Different from acute hospital setting because patient is medically stable enough and cleared for intensive, inpatient rehabilitation
- Patients receive large doses of therapy each day and can receive services from multiple disciplines including PT, OT, and SLP

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### INPATIENT

- Offer intensive therapy services, typically for those who have undergone major surgeries, SCI, TBI, or significant medical events like strokes
  - For concentrated and specialized care
  - For long-term functional goals and community reintegration
  - Restoration of mobility and ADLs
- Patients:
  - Patients with complex medical conditions
  - Patients recovering from significant injuries or illnesses
  - Elderly patients
- May also be suggested for those who live alone or have limited family or support system
- Receives around-the-clock care and therapy
  - Up to 3 hours of direct care per day, including 1.5 hours of PT and 1.5 hours of OT
- Less freedom because of its restrictiveness as the patients must adhere to the facility's schedule
- More expensive than outpatient services

# WEEK 8: INTRO TO PT & MED TERM

## PHYSICAL THERAPY SETTING II (HOMECARE, SNF, SCHOOL)

### HOME HEALTH CARE

- Goal: promote optimal functioning within the patient's living environment
  - Elderly or frail individuals
  - Post-operative patients
  - With chronic disease
- Personalized care
  - Ensures interventions are directly applicable to daily activities, promoting function independence
- Provides convenience for patients who have difficulty traveling to outpatient clinics due to mobility issues
  - Eliminates barriers to access and encourages consistent participation in rehabilitation process
- PTs can work closely with family members
  - This collaboration fosters a supportive environment where everyone can learn effective techniques
- Space limitation
  - Not all homes are equipped with ample space for comprehensive exercise routines or the use of larger equipment typically found in clinics
- Absence of specialized equipment that is readily available in clinical settings

- PT's coordinate with parents, teachers, and administrators to help students
- Supports the child in an academic environment
- Can be done as a classroom-wide push-in session, or in one-on-one or group session
- Ensure students can receive the supports they need in the least restrictive environment
- Patients:
  - Students with congenital birth defects, spina bifida, amputations and neurological disorders

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### SKILLED NURSING FACILITIES

- Also known as sub-acute rehab
- Provides round-the-clock medical care and rehab services
  - Need more support than homecare but don't require hospitalization
- Includes PTs, OTs, SLPs, and RNs
- Transitional setting for patients discharged from hospitals but still require medical supervision and rehab therapy before returning home
- Includes short-term and long-term rehab

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### SCHOOL

- Goal: help a child physically access school as independently as possible
- Services are provided under Individuals with Disability Education Act (IDEA law)
  - Part of students individualized education program (IEP)

# WEEK 8: INTRO TO PT & MED TERM

## PHYSICAL THERAPY SETTING III (WELLNESS, COMMUNITY, SPORTS)

### WELLNESS

- For clients who may not have a current injury but want to prevent dysfunction
- Overlaps with personal training, but with clinical expertise of a PT
  - Post-rehab fitness program
  - Fall prevention
  - Weight loss program
  - Chronic disease management (diabetes, hypertension)

### COMMUNITY

- Non-clinical setting
- For education, early intervention, and disability support
- May serve rural, low-income, or special needs populations
  - Home health PT
  - Public health PT campaigns
  - Mobile rehab services in rural area

### SPORTS

- Highly-sought after position
- High focus performance enhancement and return-to-sport protocols
- May involve on-field coverage, rehab clinics, or working with athletic teams
  - ACL rehab
  - Strength and conditioning

# WEEK 8: INTRO TO PT & MED TERM

## MY CHOSEN PHYSICAL THERAPY SETTING: A CASE REFLECTION

The main reason why I took a health-allied course is because I was heavily inspired by the doctors that helped my grandmother for the past years to endure her illness. Ever since I was a kid, we always go to the hospital for her monthly check-ups since she has Type I diabetes mellitus and peripheral neuropathy. As I grew older, I was captivated to pursue physical therapy, most specifically, outpatient PT since it will allow me to witness progress over time, which is both rewarding and fulfilling at the same time. One thing I also like about outpatient PT is the flexibility in terms of scheduling therapy sessions around personal commitment.

The most common patient cases that can be seen in this PT setting are musculoskeletal issues and orthopedic injuries, including ACL and RCT repair, neck pain, back pain, LBP, and osteoarthritis; sports injuries, including sprains or strains; and neurological diseases, including mild stroke, cerebral palsy, multiple sclerosis, and more.

I personally think that every single PT core responsibility must be present in every single setting. One must attain social responsibility, professional duty, accountability, altruism, compassion, integrity, and excellence to attain the high and optimum quality of care available for the patient. Without these, a PT won't be able to perform effective manual therapies, including soft tissue mobilization, joint mobilization, and trigger point release; exercise programs, including therapeutic exercises; use of treatment devices, such as TENS and ultrasound; and lastly, providing PT education for the patient. Absence of the mentioned PT core values will cause lengthening of prognosis of the patient, which could lead to worsening of patient's condition.

A 39-year-old female housewife is experiencing intense low back pain lasting over a year, and worsened by house chores and prolonged stress. Problems, such as sleep fragmentation, declining function, dysfunctional behavior, and sudden mood changes to anger, are present. Based on reports, the assessment will focus on McGill Pain Questionnaire Short Form to evaluate the pain intensity related to ongoing pain perception, Minnesota Multiphasic Personality Inventory (MMPI), or Million Behavioral Health Inventory for a semistructured interview with a psychologist to evaluate the physiologic state of the patient, cognitive behavioral techniques, which includes cognitive restructuring, problem solving, distraction, and relapse prevention to help patient notice and modify the negative thought patterns that contribute to ongoing pain and affective distress, and lastly, postural assessment. The treatment plan includes patient education on chronic pain, heat and cold modality, TENS, relaxation training, which includes breathing techniques and mindfulness-based stress reduction, and home exercise program. Treatment plan will also include taking 12.5/25 mg of Rofecoxib (Vioxx) daily for acute pain management, 5/10 mg of Zolpidem (Ambien) for initiation of sleep. In case of early morning awakening with neuropathic pain, the patient must take 25/50-100 mg of Amitriptyline (Elavil). As for the short-term goal, the patient will be able to reduce low back pain within 7 days, with regular sleep for 6 hours per night, and decreased neuropathic disturbance in the morning.

## **WEEK 8: INTRO TO PT & MED TERM**

Based on this scenario, applying rehabilitative interdisciplinary team approach model will be helpful with the provided care by maximizing independent physical function, improving psychosocial state, and returning patients to work and previous leisure pursuits, as well as maximizing patients' reintegration into community and subsequent improvement of general quality of life.